

TREATMENT PLAN

Treatment Plan: (Initial TP)
04/02/2026 - 04/02/2027

Primary Clinician: **Marleigh Johnson**

Expected Length of Treatment
(days): **365**

Problem: Substance Use Disorders

Diagnostic Impression:

04/02/2026 - [F14.20] Cocaine use disorder, Severe

04/02/2026 - [F10.20] Alcohol use disorder, Severe

04/02/2026 - [F13.20] Sedative, hypnotic, or anxiolytic use disorder, Severe

04/02/2026 - [F19.20] Other (or unknown) substance use disorder, Severe

Behavioral Definitions:

04/02/2026 - Continues substance use despite knowledge of experiencing persistent physical, legal, financial, vocational, social, and/or relationship problems that are directly caused by the use of the substance.

Client Goal: Client will increase understanding of addiction, treatment expectations, and program structure to support engagement and successful participation in treatment.

Objective: Client will attend all scheduled appointments, actively participate in individual and group sessions, and meet with their primary therapist to collaboratively develop a comprehensive Master Treatment Plan.

Intervention: Therapist will support the development of therapeutic rapport by creating a safe, nonjudgmental space for open communication, encouraging client engagement, and facilitating introductions to peers and staff to promote connection within the treatment community.

05/02/2026: **Status** - (Open)

05/16/2026: **Status** - (Attained) Client has demonstrated consistent attendance in programming and remains actively engaged in both group and individual sessions. While participation is generally appropriate, engagement is intermittently impacted by emotional distress related to family dynamics and trauma processing. Client has successfully collaborated with primary therapist to establish and maintain a comprehensive treatment plan and continues to benefit from structured therapeutic support to sustain engagement.

Objective: Client will begin to build therapeutic rapport with their therapist, staff, and peers, and demonstrate improved insight into addiction through psychoeducation over the course of the treatment plan period.

Intervention: Primary therapist will provide psychoeducation on addiction, recovery concepts, and program expectations through individual sessions and group facilitation to increase client's knowledge and insight into their substance use and treatment process.

05/02/2026: **Status** - (Open)

05/16/2026: **Status** - (Attained) Client has made measurable progress in building therapeutic rapport with staff and peers, evidenced by increased openness in group settings and willingness to process personal experiences. Insight into addiction has improved, particularly in identifying relapse triggers and environmental risk factors; however, insight remains inconsistent in application, as demonstrated by continued high-risk decision-making (i.e., employment in a bar environment). Ongoing psychoeducation and clinical support remain necessary to strengthen insight and promote behavioral change.

Treatment Plan Addendum

Level of Care:

- ☐ PHP (2.5)
- ☐ IOP 19 (2.1)
- ☒ IOP 5 (2.1)
- ☐ IOP 3 (2.1)
- ☐ OP (1.0)

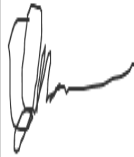
Treatment Interventions to Support Established Treatment Plan:

- ☐ Individual Sessions
 - ☒ 1x per week
 - ☐ 2x per week
 - ☐ Other:
- ☐ Group Sessions
 - ☒ 5x per week
 - ☐ 4x per week
 - ☐ 3x per week
 - ☐ 2x per week
 - ☐ 1x per week
 - ☐ Other:



April 02, 2026 09:50 AM

Staff 



Caleb Madison, BS
April 02, 2026 09:50 AM

Reviewer



Marleigh Johnson, MS, LPC
April 02, 2026 09:50 AM

Treatment Plan: **04/24/2026 - 04/24/2027**

Primary Clinician: **Marleigh Johnson**

Expected Length of Treatment (days): **365**

Problem: Substance Use Disorders

Diagnostic Impression:

04/24/2026 - [F14.20] Cocaine use disorder, Severe

04/24/2026 - [F10.20] Alcohol use disorder, Severe

04/24/2026 - [F19.20] Other (or unknown) substance use disorder, Severe

04/24/2026 - [F13.20] Sedative, hypnotic, or anxiolytic use disorder, Severe

Behavioral Definitions:

04/24/2026 - Client demonstrates a pattern of compulsive substance use (including alcohol, cocaine, benzodiazepines, and other substances) characterized by loss of control, increased tolerance, and continued use despite significant negative consequences, including legal issues, relationship strain, and medical complications.

04/24/2026 - Client exhibits high relapse risk behaviors, including impulsivity, poor decision-making, environmental vulnerability (peer influence, boredom), and inconsistent engagement in recovery supports, contributing to repeated relapse episodes following periods of sobriety.

04/24/2026 - Client demonstrates impaired functioning related to substance use, including emotional dysregulation, difficulty maintaining stability in relationships and daily responsibilities, and reliance on substances as a maladaptive coping mechanism to manage internal distress.

Client Goal: Client will increase overall functioning by addressing co-dependent patterns, processing significant past experiences (including overdose), and building a structured, value-driven lifestyle.

Objective: [REDACTED] will identify and modify at least 3 co-dependent behaviors and demonstrate use of healthy boundaries in relationships.

Intervention: Therapist will utilize DBT interpersonal effectiveness skills and psychoeducation on co-dependency to support boundary-setting, assertive communication, and reduction of maladaptive relational patterns.

06/23/2026: Status - (Open)

Objective: [REDACTED] will develop and implement a structured routine and complete at least 2 steps toward educational or vocational goals (e.g., exploring school, job applications)

Intervention: Therapist will assist [REDACTED] in creating a structured weekly schedule and utilize solution-focused and ACT-based interventions to support goal setting, accountability, and follow-through on education/employment tasks.

06/23/2026: Status - (Open)

Objective: [REDACTED] will engage in group and individual sessions by processing relationship dynamics, emotional responses, and past experiences (including overdose), and will demonstrate application of learned skills (e.g., boundaries, distress tolerance) in real-life situations.

Intervention: [REDACTED] will engage in group and individual sessions by processing relationship dynamics, emotional responses, and past experiences (including overdose), and will demonstrate application of learned skills (e.g., boundaries, distress tolerance) in real-life situations.

06/23/2026: Status - (Open)

Client Goal: [REDACTED] will develop sustained recovery by increasing acceptance of substance use disorder (Step 1), improving emotional regulation, and utilizing effective coping strategies to reduce relapse risk.

Objective: [REDACTED] will verbalize acceptance of powerlessness over substances and identify at least 3 personal consequences demonstrating unmanageability within 30 days.

Intervention: Therapist will utilize Motivational Interviewing (MI) and CBT techniques to explore ambivalence, challenge denial, and process Step 1 concepts, linking [REDACTED]'s substance use history to patterns of loss of control and unmanageability.

06/23/2026: Status - (Open)

Objective: [REDACTED] will identify and implement at least 3 coping skills to manage cravings, emotional distress, and relapse triggers (e.g., boredom, relationships, impulsivity).

Intervention: Therapist will provide CBT-based relapse prevention and DBT-informed coping skills training (e.g., distress tolerance, urge surfing), and rehearse application of skills to identified high-risk situations.

06/23/2026: Status - (Open)

Objective: [REDACTED] will actively participate in both group and individual therapy by discussing relapse triggers, sharing personal experiences related to substance use, and applying feedback from clinicians and peers to support recovery-oriented behavior.

Intervention: Therapist will facilitate processing in both group and individual sessions, encouraging [REDACTED] to share experiences, receive peer feedback, and integrate insights into individualized relapse prevention planning and 12-step engagement.

06/23/2026: Status - (Open)

Treatment Plan Addendum

Level of Care:

- ☐ PHP (2.5)
- ☐ IOP 19 (2.1)

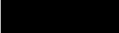
- ☒ IOP 5 (2:1)
☐ IOP 3 (2:1)
☐ OP (1:0)

Treatment Interventions to Support Established Treatment Plan:

- ☒ Individual Sessions
☒ 1x per week
☐ 2x per week
☐ Other:
☒ Group Sessions
☒ 5x per week
☐ 4x per week
☐ 3x per week
☐ 2x per week
☐ 1x per week
☐ Other:




April 24, 2026 08:35 AM

Staff 


Marleigh Johnson, MS, LPC

April 24, 2026 08:35 AM

Treatment Plan: **04/24/2026 -**
04/24/2027

Primary Clinician: **Marleigh Johnson**

Expected Length of Treatment
(days): **365**

Problem: Substance Use Disorders**Diagnostic Impression:**

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04/24/2026 - Client exhibits high relapse risk behaviors, including impulsivity, poor decision-making, environmental vulnerability (peer influence, boredom), and inconsistent engagement in recovery supports, contributing to repeated relapse episodes following periods of sobriety.

04/24/2026 - Client demonstrates impaired functioning related to substance use, including emotional dysregulation, difficulty maintaining stability in relationships and daily responsibilities, and reliance on substances as a maladaptive coping mechanism to manage internal distress.

Client Goal: Client will increase overall functioning by addressing co-dependent patterns, processing significant past experiences (including overdose), and building a structured, value-driven lifestyle.

Objective:  will identify and modify at least 3 co-dependent behaviors and demonstrate use of healthy boundaries in relationships.

Intervention: Therapist will utilize DBT interpersonal effectiveness skills and psychoeducation on co-dependency to support boundary-setting, assertive communication, and reduction of maladaptive relational patterns.

06/23/2026: Status - (Open)

07/15/2026: Status - (Extended) Client has begun identifying co-dependent patterns, particularly within family and interpersonal relationships, and has demonstrated emerging awareness of maladaptive behaviors. She has engaged in skill-building related to boundary setting, though implementation remains inconsistent due to emotional reactivity and difficulty managing interpersonal stressors. Continued focus on DBT interpersonal effectiveness skills is clinically indicated to support sustained behavioral change.

Objective:  will develop and implement a structured routine and complete at least 2 steps toward educational or vocational goals (e.g., exploring school, job applications)

Intervention: Therapist will assist  in creating a structured weekly schedule and utilize solution-focused and ACT-based interventions to support goal setting, accountability, and follow-through on education/employment tasks.

06/23/2026: Status - (Open)

07/15/2026: Status - (Extended) Client has taken steps toward vocational engagement, including obtaining employment; however, current employment in a bar setting presents a significant relapse risk due to direct exposure to substances. Client continues to struggle with maintaining a consistent daily structure outside of treatment programming, with difficulties in follow-through and routine stabilization. Ongoing support is required to assist client in developing a recovery-oriented schedule that reduces risk and promotes stability.

Objective: ■ will engage in group and individual sessions by processing relationship dynamics, emotional responses, and past experiences (including overdose), and will demonstrate application of learned skills (e.g., boundaries, distress tolerance) in real-life situations.

Intervention: ■ will engage in group and individual sessions by processing relationship dynamics, emotional responses, and past experiences (including overdose), and will demonstrate application of learned skills (e.g., boundaries, distress tolerance) in real-life situations.

06/23/2026: Status - (Open)

07/15/2026: Status - (Extended) Client is actively engaging in both group and individual therapy, processing trauma related to past overdose and ongoing relational stressors. She demonstrates insight into emotional triggers and has begun applying coping strategies; however, skill application remains inconsistent in real-world situations, particularly when experiencing heightened emotional distress. Continued therapeutic intervention is needed to increase generalization and independent use of learned skills.

Client Goal: ■ will develop sustained recovery by increasing acceptance of substance use disorder (Step 1), improving emotional regulation, and utilizing effective coping strategies to reduce relapse risk.

Objective: ■ will verbalize acceptance of powerlessness over substances and identify at least 3 personal consequences demonstrating unmanageability within 30 days.

Intervention: Therapist will utilize Motivational Interviewing (MI) and CBT techniques to explore ambivalence, challenge denial, and process Step 1 concepts, linking ■'s substance use history to patterns of loss of control and unmanageability.

06/23/2026: Status - (Open)

07/15/2026: Status - (Extended) Client has begun acknowledging the impact of her substance use and can identify consequences related to relapse, including overdose history and interpersonal strain. While she demonstrates increasing awareness, she continues to exhibit ambivalence and remains in the contemplation stage, as evidenced by ongoing engagement in high-risk environments. Continued motivational interviewing is required to support movement toward full acceptance and commitment to recovery.

Objective: ■ will identify and implement at least 3 coping skills to manage cravings, emotional distress, and relapse triggers (e.g., boredom, relationships, impulsivity).

Intervention: Therapist will provide CBT-based relapse prevention and DBT-informed coping skills training (e.g., distress tolerance, urge surfing), and rehearse application of skills to identified high-risk situations.

06/23/2026: Status - (Open)

07/15/2026: Status - (Extended) Client has identified several coping strategies, including emotional regulation and distress tolerance skills, and is able to verbalize their use in session. However, implementation remains inconsistent outside of structured settings, particularly in response to triggers such as boredom, emotional distress, and environmental exposure. Client continues to report moderate cravings, indicating a need for ongoing skill reinforcement and relapse prevention planning.

Objective: ■ will actively participate in both group and individual therapy by discussing relapse triggers, sharing personal experiences related to substance use, and applying feedback from clinicians and peers to support recovery-oriented behavior.

Intervention: Therapist will facilitate processing in both group and individual sessions, encouraging ■ to share experiences, receive peer feedback, and integrate insights into individualized relapse prevention planning and 12-step engagement.

06/23/2026: Status - (Open)

07/15/2026: Status - (Extended) Client is actively participating in treatment and demonstrates willingness to share personal experiences, particularly related to relapse history, trauma, and interpersonal challenges. She is receptive to feedback and demonstrates growing ability to integrate peer and clinician input into her recovery process. Despite this progress, ongoing emotional dysregulation and environmental risk factors continue to interfere with consistent behavioral change, indicating the need for continued structured care.

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■ ■ ■

April 24, 2026 08:35 AM

Staff ■ ■ ■

Marleigh Johnson, MS, LPC

Marleigh Johnson, MS, LPC

April 24, 2026 08:35 AM

Treatment Plan: 04/24/2026 -
04/24/2027

Primary Clinician: Marleigh Johnson

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(days): 365

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06/23/2026: Status - (Open)

07/15/2026: Status - (Extended) Client has begun identifying co-dependent patterns, particularly within family and interpersonal relationships, and has demonstrated emerging awareness of maladaptive behaviors. She has engaged in skill-building related to boundary setting, though implementation remains inconsistent due to emotional reactivity and difficulty managing interpersonal stressors. Continued focus on DBT interpersonal effectiveness skills is clinically indicated to support sustained behavioral change.

08/08/2026: Status - (Extended) Client continues to demonstrate growing awareness of co-dependent behaviors and relational patterns. She has engaged in boundary-setting skill development; however, implementation remains inconsistent, particularly during emotional or interpersonal distress. Continued outpatient support is required to strengthen skill application and promote stability in relationships.

Objective: ■ will develop and implement a structured routine and complete at least 2 steps toward educational or vocational goals (e.g., exploring school, job applications)

Intervention: Therapist will assist ■ in creating a structured weekly schedule and utilize solution-focused and ACT-based interventions to support goal setting, accountability, and follow-through on education/employment tasks.

06/23/2026: Status - (Open)

07/15/2026: Status - (Extended) Client has taken steps toward vocational engagement, including obtaining employment; however, current employment in a bar setting presents a significant relapse risk due to direct exposure to substances. Client continues to struggle with maintaining a consistent daily structure outside of treatment programming, with difficulties in follow-through and routine stabilization. Ongoing support is required to assist client in developing a recovery-oriented schedule that reduces risk and promotes stability.

08/08/2026: Status - (Extended) Client has made progress in vocational engagement; however, ongoing employment in a high-risk bar environment continues to present relapse risk. She continues to struggle with maintaining a consistent, recovery-oriented routine outside of structured treatment. Ongoing clinical support is needed to assist with routine stabilization and alignment of goals with recovery needs.

Objective: ■ will engage in group and individual sessions by processing relationship dynamics, emotional responses, and past experiences (including overdose), and will demonstrate application of learned skills (e.g., boundaries, distress tolerance) in real-life situations.

Intervention: ■ will engage in group and individual sessions by processing relationship dynamics, emotional responses, and past experiences (including overdose), and will demonstrate application of learned skills (e.g., boundaries, distress tolerance) in real-life situations.

06/23/2026: Status - (Open)

07/15/2026: Status - (Extended) Client is actively engaging in both group and individual therapy, processing trauma related to past overdose and ongoing relational stressors. She demonstrates insight into emotional triggers and has begun applying coping strategies; however, skill application remains inconsistent in real-world situations, particularly when experiencing heightened emotional distress. Continued therapeutic intervention is needed to increase generalization and independent use of learned skills.

08/08/2026: Status - (Extended) Client remains actively engaged in processing trauma, relapse history, and relational dynamics in both group and individual therapy. She demonstrates insight into emotional triggers and has begun applying coping skills; however, application remains inconsistent outside of sessions. Continued treatment is necessary to support generalization and independent use of skills.

Client Goal: ■ will develop sustained recovery by increasing acceptance of substance use disorder (Step 1), improving emotional regulation, and utilizing effective coping strategies to reduce relapse risk.

Objective: ■ will verbalize acceptance of powerlessness over substances and identify at least 3 personal consequences demonstrating unmanageability within 30 days.

Intervention: Therapist will utilize Motivational Interviewing (MI) and CBT techniques to explore ambivalence, challenge denial, and process Step 1 concepts, linking ■'s substance use history to patterns of loss of control and unmanageability.

06/23/2026: Status - (Open)

07/15/2026: Status - (Extended) Client has begun acknowledging the impact of her substance use and can identify consequences related to relapse, including overdose history and interpersonal strain. While she demonstrates increasing awareness, she continues to exhibit ambivalence and remains in the contemplation stage, as evidenced by ongoing engagement in high-risk environments. Continued motivational interviewing is required to support movement toward full acceptance and commitment to recovery.

08/08/2026: Status - (Extended) Client demonstrates increasing awareness of the impact of her substance use and is able to identify consequences, including overdose and relational instability. However, she continues to exhibit ambivalence and remains in the contemplation stage, as evidenced by ongoing high-risk decision-making. Continued motivational interventions are required to strengthen commitment to recovery.

Objective: ■ will identify and implement at least 3 coping skills to manage cravings, emotional distress, and relapse triggers (e.g., boredom, relationships, impulsivity).

Intervention: Therapist will provide CBT-based relapse prevention and DBT-informed coping skills training (e.g., distress tolerance, urge surfing), and rehearse application of skills to identified high-risk situations.

06/23/2026: Status - (Open)

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08/08/2026: Status - (Extended) Client has identified multiple coping strategies and is able to verbalize their use in session. However, she continues to experience difficulty consistently implementing these skills independently, particularly in response to environmental triggers and emotional distress. Continued outpatient treatment is required to reinforce skill use and relapse prevention planning.

Objective: ■ will actively participate in both group and individual therapy by discussing relapse triggers, sharing personal experiences related to substance use, and applying feedback from clinicians and peers to support recovery-oriented behavior.

Intervention: Therapist will facilitate processing in both group and individual sessions, encouraging ■ to share experiences, receive peer feedback, and integrate insights into individualized relapse prevention planning and 12-step engagement.

06/23/2026: Status - (Open)

07/15/2026: Status - (Extended) Client is actively participating in treatment and demonstrates willingness to share personal experiences, particularly related to relapse history, trauma, and interpersonal challenges. She is receptive to feedback and demonstrates growing ability to integrate peer and clinician input into her recovery process. Despite this progress, ongoing emotional dysregulation and environmental risk factors continue to interfere with consistent behavioral change, indicating the need for continued structured care.

08/08/2026: Status - (Extended) Client continues to actively participate in treatment and demonstrates openness to sharing personal experiences and receiving feedback. She shows emerging ability to integrate feedback into her recovery; however, emotional dysregulation and environmental risk factors continue to interfere with consistent behavioral change. Continued support is needed to strengthen application and sustain progress.



April 24, 2026 08:35 AM

Staff



Marleigh Johnson, MS, LPC
April 24, 2026 08:35 AM

----- END OF TX PLAN -----

TREATMENT PLAN REVIEW

TPR Overview

Created By

Marleigh Johnson, MS, LPC

Date

06/05/2026

Master Problem List

Diagnostic Impression	Problem	EST Completed	Date Resolved
[F10.20] Alcohol use disorder, Severe Secondary	- Substance Use Disorders	04/24/2027	--
[F13.20] Sedative, hypnotic, or anxiolytic use disorder, Severe Other	- Substance Use Disorders	04/24/2027	--
[F14.20] Cocaine use disorder, Severe Primary	- Substance Use Disorders	04/24/2027	--
[F19.20] Other (or unknown) substance use disorder, Severe Tertiary	- Substance Use Disorders	04/24/2027	--

Discharge / Transfer

Anticipated Discharge Date:

08/31/2026

Living Arrangements:

█ is currently residing at sober living (R3 House) and reports overall stability in housing. While the environment provides basic structure, the client continues to encounter triggers within her surroundings, including exposure to high-risk settings and interpersonal stressors. She demonstrates some awareness of these risks but continues to require support in maintaining boundaries and making recovery-oriented housing decisions. Continued outpatient support is necessary to assist the client in stabilizing within her environment while reducing relapse risk

Education:

Client will continue working toward vocational and educational goals, including maintaining employment and exploring additional structured opportunities. Clinical focus remains on supporting the client in engaging in prosocial, recovery-oriented activities while addressing the risks associated with current employment in a high-risk bar environment.

Therapy(Specify individual, family or group treatment):

Client will continue participation in OP programming, including group therapy, individual therapy, and recommended family sessions. Treatment will focus on relapse prevention, emotional regulation, trauma processing, co-dependency, and development of healthy coping strategies.

Discharge Transition Obstacles:

Although the client has made sufficient progress to transition to outpatient care, several ongoing barriers to sustained recovery remain. These include continued emotional dysregulation, inconsistent application of coping strategies, and engagement in high-risk environments (e.g., employment setting). Additionally, the client continues to demonstrate co-dependent relational patterns and difficulty maintaining boundaries, which contribute to instability and relapse risk. Without continued outpatient structure and therapeutic support, the client remains at risk for regression and return to use.

Follow up Appointments:

Client will be encouraged to maintain consistent engagement in outpatient therapy, medication management, and recovery supports upon discharge. Continued participation in 12-step programming is recommended, including obtaining a sponsor to increase accountability and support recovery maintenance.

Treatment Plan Review Note

This treatment plan update reflects █'s step-down from IOP to outpatient level of care, consisting of two group sessions per week effective 6/6.

Since the last review, the client has continued to engage in treatment and has maintained participation in both group and individual therapy. She has demonstrated improved insight into her substance use disorder, including identification of relapse triggers, increased awareness of maladaptive relational patterns, and willingness to engage in therapeutic processing. This progress supports the appropriateness of transitioning to a lower level of care.

Despite these gains, the client continues to present with clinically significant needs that require ongoing therapeutic support. She continues to experience emotional dysregulation, difficulty consistently applying coping strategies, and vulnerability to interpersonal stressors, all of which impact her ability to maintain stability independently. While she is able to verbalize coping skills and recovery concepts, implementation outside of structured treatment remains inconsistent, particularly in response to emotional distress, boredom, and environmental triggers.

A continued area of concern is the client's ongoing employment in a bar environment, which exposes her to substances and high-risk social dynamics. This setting continues to significantly increase relapse risk and reflects ongoing ambivalence in recovery. Additionally, the client continues to demonstrate co-dependent relational patterns and difficulty maintaining healthy boundaries, further contributing to emotional instability and relapse vulnerability.

The client's recovery environment, while partially supportive, continues to present challenges as she adjusts to increased independence. She requires ongoing support to establish a consistent routine, strengthen recovery-oriented decision-making, and build sustainable supports outside of treatment. She continues to report cravings at times and identifies multiple triggers that require continued clinical intervention.

At the outpatient level of care, the client continues to benefit from structured support, accountability, and skill reinforcement. OP remains medically necessary to provide ongoing relapse prevention, support continued development and generalization of coping skills, address co-occurring mental health symptoms, and assist the client in managing high-risk environmental exposures.

Treatment will continue to focus on strengthening emotional regulation, increasing consistency in skill application, reducing environmental risk factors, improving boundary setting, and supporting the development of a stable, recovery-oriented lifestyle.

Next Review Due

08/08/2026

ASAM Placement

ASAM Placement Summary Sheet

Date:

2026-06-09

Type (Check One):



Admission



Continued Stay



Discharge

Dimension

	Level of Risk	Level of Care	Criteria Included/Comments
D1. Acute intoxication and/or withdrawal potential:	1	1.0	Client is not currently experiencing acute withdrawal symptoms and has maintained abstinence within a structured environment. However, her extensive history of polysubstance use, including alcohol, cocaine, benzodiazepines, and other substances, combined with multiple prior overdose events, places her at elevated risk in the event of relapse. Given her current exposure to high-risk environments and ongoing moderate cravings, continued clinical monitoring is necessary to ensure early identification of destabilization and to prevent progression to acute intoxication or withdrawal.
D2. Biomedical Conditions and Complications:	1	1.0	There are no current acute biomedical concerns; however, the client's history of multiple overdose events represents a chronic medical vulnerability. Additionally, she is actively processing past overdose experiences, which contributes to emotional activation and potential somatic stress. While stable at present, the risk of medical complications remains elevated in the context of relapse, necessitating ongoing monitoring within a structured level of care.

	Level of Risk	Level of Care	Criteria Included/Comments
D3. Emotional/Behavioral or cognitive conditions and complications:	2	1.0	Client presents with significant co-occurring psychiatric conditions, including Bipolar Disorder, Generalized Anxiety Disorder, Major Depressive Disorder, and ADHD. These conditions contribute to persistent emotional dysregulation, impulsivity, impaired distress tolerance, and difficulty maintaining behavioral consistency. She is actively processing trauma, including prior overdose experiences, which increases emotional vulnerability and contributes to guilt, shame, and reactivity. Client continues to struggle with regulating emotional responses, particularly in relation to family dynamics and interpersonal stressors. Her recent decision to engage in employment within a bar environment, despite awareness of relapse risk, further reflects impaired judgment and limited application of insight. Severity in this dimension strongly supports the need for continued structured therapeutic intervention.
D4. Readiness to change:	2	1.0	Client demonstrates engagement in treatment and verbalizes motivation for recovery; however, she remains in the contemplation stage of change. Although she is able to identify relapse risks and articulate recovery goals, her behavior reflects ongoing ambivalence, as evidenced by continued participation in high-risk activities, including employment in a bar setting. External structure, accountability, and therapeutic intervention remain primary motivators for engagement, indicating that she has not yet internalized recovery-oriented behaviors. Continued IOP care is required to support progression toward sustained commitment to change.
D5. Relapse, continued use or continued problem potential:	3	1.0	Client remains at high to severe risk of relapse, supported by a history of chronic relapse following periods of stability, prior overdose events, impulsivity, and difficulty managing emotional distress. She identifies multiple ongoing triggers, including boredom, lack of structure, interpersonal conflict, and family-related stressors. A critical escalation in relapse risk is her current employment in a bar environment, which provides constant exposure to alcohol, normalization of substance use, and direct access to substances. Combined with her inconsistent application of coping skills outside of structured settings and ongoing moderate cravings, the client requires continued intensive support to prevent return to use.
D6. Recovery Environment:	2	2.1	Client currently resides in a structured sober living environment, which provides accountability and peer support and serves as a protective factor. However, the effectiveness of this environment is limited by her difficulty maintaining routine, navigating interpersonal dynamics, and managing increased independence associated with employment. The addition of a high-risk occupational setting further compromises her recovery environment by introducing consistent exposure to substances. While supports are present, the overall environment remains unstable enough to warrant continued clinical oversight at the IOP level.

Indicate level of care recommended:

1.0

Indicate level of care received:

1.0

If recommended level of care is different from received, why?

n/a

Indicate the program or Facility referred to:

R3 Recovery Services

Brief Assessment of Recovery Capital (BARC-10)

Brief Assessment of Recovery Capital (BARC-10)

Instructions: On a scale of 1 (Strongly disagree) to 6 (Strongly agree), please indicate your level of agreement with the following statements.

1. There are important things to me in life than using substances.

☐ 1 Strongly Disagree
 ☐ 2 Disagree
 ☐ 3 Somewhat Disagree
 ☐ 4 Somewhat Agree
 ☒ 5 Agree
 ☐ 6 Strongly Agree

2. In general I am happy with my life.

☐ 1 Strongly Disagree
 ☐ 2 Disagree
 ☐ 3 Somewhat Disagree
 ☒ 4 Somewhat Agree
 ☐ 5 Agree
 ☐ 6 Strongly Agree

3. I have enough energy to complete the tasks I set myself.

☐ 1 Strongly Disagree
 ☐ 2 Disagree
 ☐ 3 Somewhat Disagree
 ☒ 4 Somewhat Agree
 ☐ 5 Agree
 ☐ 6 Strongly Agree

4. I am proud of the community I live in and feel part of it.

☐ 1 Strongly Disagree
 ☐ 2 Disagree
 ☐ 3 Somewhat Disagree
 ☒ 4 Somewhat Agree
 ☐ 5 Agree
 ☐ 6 Strongly Agree

5. I get lots of support from friends.

☐ 1 Strongly Disagree
 ☐ 2 Disagree
 ☐ 3 Somewhat Disagree
 ☐ 4 Somewhat Agree
 ☒ 5 Agree
 ☐ 6 Strongly Agree

6. I regard my life as challenging and fulfilling without the need for using drugs or alcohol.

☐ 1 Strongly Disagree
 ☐ 2 Disagree
 ☐ 3 Somewhat Disagree
 ☒ 4 Somewhat Agree
 ☐ 5 Agree
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7. My living space has helped to drive my recovery journey.

☐ 1 Strongly Disagree
 ☐ 2 Disagree
 ☐ 3 Somewhat Disagree
 ☒ 4 Somewhat Agree
 ☐ 5 Agree
 ☐ 6 Strongly Agree

8. I take full responsibility for my actions.

☐ 1 Strongly Disagree
 ☐ 2 Disagree
 ☒ 3 Somewhat Disagree
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9. I am happy dealing with a range of professional people.

☐ 1 Strongly Disagree
 ☐ 2 Disagree
 ☒ 3 Somewhat Disagree
 ☐ 4 Somewhat Agree
 ☐ 5 Agree
 ☐ 6 Strongly Agree

10. I am making good progress on my recovery journey.

☐ 1 Strongly Disagree
 ☐ 2 Disagree
 ☐ 3 Somewhat Disagree
 ☒ 4 Somewhat Agree
 ☐ 5 Agree
 ☐ 6 Strongly Agree

Scoring

Total scores can range from a minimum of 10 to a maximum of 60. A ROC Curve Analysis showed the BARC-10 had predictive validity with sustained remission (i.e., 1 year or more) using a cut-off score of 47 in a sample whose average length of recovery time was 7 years.

42

Generalized Anxiety Disorder 7-item (GAD-7) Scale

GAD-7

Over the last 2 weeks, how often have you been bothered by the following problems?

1. Feeling nervous, anxious or on edge

☒ Not at all
 ☐ Several days
 ☐ More than half the days
 ☐ Nearly every day

2. Not being able to stop or control worrying

☒ Not at all
 ☐ Several days
 ☐ More than half the days
 ☐ Nearly every day

3. Worrying too much about different things

☒ Not at all
 ☐ Several days
 ☐ More than half the days
 ☐ Nearly every day

4. Trouble relaxing

- ☒ Not at all ☐ Several days ☐ More than half the days ☐ Nearly every day

5. Being so restless that it is hard to sit still

- ☒ Not at all ☐ Several days ☐ More than half the days ☐ Nearly every day

6. Becoming easily annoyed or irritable

- ☐ Not at all ☒ Several days ☐ More than half the days ☐ Nearly every day

7. Feeling afraid as if something awful might happen

- ☐ Not at all ☒ Several days ☐ More than half the days ☐ Nearly every day

GAD7 Score:

4

If you checked off any problems, how difficult have these problems made it for you to do your work, take care of things at home, or get along with other people?

- ☐ Not difficult at all ☒ Somewhat difficult ☐ Very difficult ☐ Extremely difficult

Patient Health Questionnaire (PHQ-9)

PATIENT HEALTH QUESTIONNAIRE (PHQ-9)

Over the last 2 weeks, how often have you been bothered by any of the following problems?

1. Little interest or pleasure in doing things.

- ☒ Not at all ☐ Several Days ☐ More than half the days ☐ Nearly every day

2. Feeling down, depressed, or hopeless.

- ☒ Not at all ☐ Several Days ☐ More than half the days ☐ Nearly every day

3. Trouble falling asleep, staying asleep, or sleeping too much.

- ☒ Not at all ☐ Several Days ☐ More than half the days ☐ Nearly every day

4. Feeling tired or having little energy.

- ☐ Not at all ☒ Several Days ☐ More than half the days ☐ Nearly every day

5. Poor appetite or overeating.

- ☐ Not at all ☒ Several Days ☐ More than half the days ☐ Nearly every day

6. Feeling bad about yourself - or that you are a failure or have let yourself or your family down

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7. Trouble concentrating on things such as reading the newspaper or watching television.

- ☒ Not at all ☐ Several Days ☐ More than half the days ☐ Nearly every day

8. Moving or speaking so slowly that other people could have noticed? Or the opposite - being so fidgety or restless that you have been moving around a lot more than usual

- ☒ Not at all ☐ Several Days ☐ More than half the days ☐ Nearly every day

9. Thoughts you would be better off dead or of hurting yourself in some way

- ☒ Not at all ☐ Several Days ☐ More than half the days ☐ Nearly every day

PHQ-9 Score:

2

10. If you checked off any problems, how difficult have these problems made it for you to do your work, take care of things at home, or get along with other people?

☒ Not difficult at all ☐ Somewhat difficult ☐ Very difficult ☐ Extremely difficult

Treatment Plan LOC addendum

Level of Care

☐ PHP ☐ IOP 5 ☐ IOP 3 ☒ OP ☐ Other:

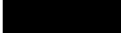
Group sessions

☐ 5 Groups per week ☐ 4 Groups per week ☐ 3 Groups per week ☒ 2 Groups per week ☐ 1 Group per week ☐ Other:

Individual sessions

☒ 1 session per week ☐ 2 sessions per week ☐ Other:

Jun 9 2026 7:51AM

Staff 

Marleigh Johnson, MS, LPC

Jun 9 2026 7:51AM

Treatment Plan: 04/24/2026 - 04/24/2027

Problem: Substance Use Disorders**Diagnostic Impression:**

04/24/2026 - [F14.20] Cocaine use disorder, Severe

04/24/2026 - [F10.20] Alcohol use disorder, Severe

04/24/2026 - [F19.20] Other (or unknown) substance use disorder, Severe

04/24/2026 - [F13.20] Sedative, hypnotic, or anxiolytic use disorder, Severe

Behavioral Definitions:**04/24/2026** - Client demonstrates a pattern of compulsive substance use (including alcohol, cocaine, benzodiazepines, and other substances) characterized by loss of control, increased tolerance, and continued use despite significant negative consequences, including legal issues, relationship strain, and medical complications.**04/24/2026** - Client exhibits high relapse risk behaviors, including impulsivity, poor decision-making, environmental vulnerability (peer influence, boredom), and inconsistent engagement in recovery supports, contributing to repeated relapse episodes following periods of sobriety.**04/24/2026** - Client demonstrates impaired functioning related to substance use, including emotional dysregulation, difficulty maintaining stability in relationships and daily responsibilities, and reliance on substances as a maladaptive coping mechanism to manage internal distress.**Client Goal:** Client will increase overall functioning by addressing co-dependent patterns, processing significant past experiences (including overdose), and building a structured, value-driven lifestyle.**Objective:**  will identify and modify at least 3 co-dependent behaviors and demonstrate use of healthy boundaries in relationships.**Intervention:** Therapist will utilize DBT interpersonal effectiveness skills and psychoeducation on co-dependency to support boundary-setting, assertive communication, and reduction of maladaptive relational patterns.**06/23/2026: Status** - (Open)**07/15/2026: Status** - (Extended) Client has begun identifying co-dependent patterns, particularly within family and interpersonal relationships, and has demonstrated emerging awareness of maladaptive behaviors. She has engaged in skill-building related to boundary setting, though implementation remains inconsistent due to emotional reactivity and difficulty managing interpersonal stressors. Continued focus on DBT interpersonal effectiveness skills is clinically indicated to support sustained behavioral change.**08/08/2026: Status** - (Extended) Client continues to demonstrate growing awareness of co-dependent behaviors and relational patterns. She has engaged in boundary-setting skill development; however, implementation remains inconsistent, particularly during emotional or interpersonal distress. Continued outpatient support is required to strengthen skill application and promote stability in relationships.**Objective:**  will develop and implement a structured routine and complete at least 2 steps toward educational or vocational goals (e.g., exploring school, job applications)**Intervention:** Therapist will assist  in creating a structured weekly schedule and utilize solution-focused and ACT-based interventions to support goal setting, accountability, and follow-through on education/employment tasks.**06/23/2026: Status** - (Open)

07/15/2026: Status - (Extended) Client has taken steps toward vocational engagement, including obtaining employment; however, current employment in a bar setting presents a significant relapse risk due to direct exposure to substances. Client continues to struggle with maintaining a consistent daily structure outside of treatment programming, with difficulties in follow-through and routine stabilization. Ongoing support is required to assist client in developing a recovery-oriented schedule that reduces risk and promotes stability.

08/08/2026: Status - (Extended) Client has made progress in vocational engagement; however, ongoing employment in a high-risk bar environment continues to present relapse risk. She continues to struggle with maintaining a consistent, recovery-oriented routine outside of structured treatment. Ongoing clinical support is needed to assist with routine stabilization and alignment of goals with recovery needs.

Objective: ■ will engage in group and individual sessions by processing relationship dynamics, emotional responses, and past experiences (including overdose), and will demonstrate application of learned skills (e.g., boundaries, distress tolerance) in real-life situations.

Intervention: ■ will engage in group and individual sessions by processing relationship dynamics, emotional responses, and past experiences (including overdose), and will demonstrate application of learned skills (e.g., boundaries, distress tolerance) in real-life situations.

06/23/2026: Status - (Open)

07/15/2026: Status - (Extended) Client is actively engaging in both group and individual therapy, processing trauma related to past overdose and ongoing relational stressors. She demonstrates insight into emotional triggers and has begun applying coping strategies; however, skill application remains inconsistent in real-world situations, particularly when experiencing heightened emotional distress. Continued therapeutic intervention is needed to increase generalization and independent use of learned skills.

08/08/2026: Status - (Extended) Client remains actively engaged in processing trauma, relapse history, and relational dynamics in both group and individual therapy. She demonstrates insight into emotional triggers and has begun applying coping skills; however, application remains inconsistent outside of sessions. Continued treatment is necessary to support generalization and independent use of skills.

Client Goal: ■ will develop sustained recovery by increasing acceptance of substance use disorder (Step 1), improving emotional regulation, and utilizing effective coping strategies to reduce relapse risk.

Objective: ■ will verbalize acceptance of powerlessness over substances and identify at least 3 personal consequences demonstrating unmanageability within 30 days.

Intervention: Therapist will utilize Motivational Interviewing (MI) and CBT techniques to explore ambivalence, challenge denial, and process Step 1 concepts, linking ■'s substance use history to patterns of loss of control and unmanageability.

06/23/2026: Status - (Open)

07/15/2026: Status - (Extended) Client has begun acknowledging the impact of her substance use and can identify consequences related to relapse, including overdose history and interpersonal strain. While she demonstrates increasing awareness, she continues to exhibit ambivalence and remains in the contemplation stage, as evidenced by ongoing engagement in high-risk environments. Continued motivational interviewing is required to support movement toward full acceptance and commitment to recovery.

08/08/2026: Status - (Extended) Client demonstrates increasing awareness of the impact of her substance use and is able to identify consequences, including overdose and relational instability. However, she continues to exhibit ambivalence and remains in the contemplation stage, as evidenced by ongoing high-risk decision-making. Continued motivational interventions are required to strengthen commitment to recovery.

Objective: ■ will identify and implement at least 3 coping skills to manage cravings, emotional distress, and relapse triggers (e.g., boredom, relationships, impulsivity).

Intervention: Therapist will provide CBT-based relapse prevention and DBT-informed coping skills training (e.g., distress tolerance, urge surfing), and rehearse application of skills to identified high-risk situations.

06/23/2026: Status - (Open)

07/15/2026: Status - (Extended) Client has identified several coping strategies, including emotional regulation and distress tolerance skills, and is able to verbalize their use in session. However, implementation remains inconsistent outside of structured settings, particularly in response to triggers such as boredom, emotional distress, and environmental exposure. Client continues to report moderate cravings, indicating a need for ongoing skill reinforcement and relapse prevention planning.

08/08/2026: Status - (Extended) Client has identified multiple coping strategies and is able to verbalize their use in session. However, she continues to experience difficulty consistently implementing these skills independently, particularly in response to environmental triggers and emotional distress. Continued outpatient treatment is required to reinforce skill use and relapse prevention planning.

Objective: ■ will actively participate in both group and individual therapy by discussing relapse triggers, sharing personal experiences related to substance use, and applying feedback from clinicians and peers to support recovery-oriented behavior.

Intervention: Therapist will facilitate processing in both group and individual sessions, encouraging ■ to share experiences, receive peer feedback, and integrate insights into individualized relapse prevention planning and 12-step engagement.

06/23/2026: Status - (Open)

07/15/2026: Status - (Extended) Client is actively participating in treatment and demonstrates willingness to share personal experiences, particularly related to relapse history, trauma, and interpersonal challenges. She is receptive to feedback and demonstrates growing ability to integrate peer and clinician input into her recovery process. Despite this progress, ongoing emotional dysregulation and environmental risk factors continue to interfere with consistent behavioral change, indicating the need for continued structured care.

08/08/2026: Status - (Extended) Client continues to actively participate in treatment and demonstrates openness to sharing personal experiences and receiving feedback. She shows emerging ability to integrate feedback into her recovery; however, emotional dysregulation and environmental risk factors continue to interfere with consistent behavioral change. Continued support is needed to strengthen application and sustain progress.

■■■



■■■
April 24, 2026 08:35 AM

Staff ■■■



Marleigh Johnson, MS, LPC
April 24, 2026 08:35 AM

TPR Overview

Created By

Date

Marleigh Johnson, MS, LPC

05/14/2026

Master Problem List

Diagnostic Impression	Problem	EST Completed	Date Resolved
[F10.20] Alcohol use disorder, Severe Secondary	- Substance Use Disorders	04/24/2027	--
[F13.20] Sedative, hypnotic, or anxiolytic use disorder, Severe Other	- Substance Use Disorders	04/24/2027	--
[F14.20] Cocaine use disorder, Severe Primary	- Substance Use Disorders	04/24/2027	--
[F19.20] Other (or unknown) substance use disorder, Severe Tertiary	- Substance Use Disorders	04/24/2027	--

Discharge / Transfer

Anticipated Discharge Date:

07/30/2026

Living Arrangements:

Client will continue residing in a structured sober living environment (R3 House), which provides accountability and peer support. While this environment is protective, its effectiveness is currently limited by the client's difficulty maintaining routine, interpersonal stressors, and increasing independence through employment, all of which contribute to ongoing instability and relapse risk.

Education:

Client will continue working toward vocational and educational goals, including maintaining employment and exploring additional structured opportunities. Clinical focus remains on supporting the client in engaging in prosocial, recovery-oriented activities while addressing the risks associated with current employment in a high-risk bar environment.

Therapy(Specify individual, family or group treatment):

Client will continue participation in IOP programming, including group therapy, individual therapy, and recommended family sessions. Treatment will focus on relapse prevention, emotional regulation, trauma processing, co-dependency, and development of healthy coping strategies. Family involvement will be incorporated to address relational dynamics and strengthen the client's support system, as this has been identified as a critical component of her recovery.

Discharge Transition Obstacles:

Client continues to present with significant barriers to discharge, including persistent emotional dysregulation, unresolved trauma related to prior overdose experiences, and maladaptive relational patterns characterized by co-dependency and poor boundaries. She demonstrates ongoing impulsivity and inconsistent application of coping strategies, particularly in unstructured environments. A primary and acute barrier is her recent decision to work in a bar setting, which provides constant exposure to alcohol, reinforces substance-use-associated behaviors, and significantly increases relapse risk. Combined with her history of chronic relapse, overdose, and prior medication noncompliance, these factors indicate that the client is not yet able to safely transition to a lower level of care without substantial risk of decompensation and return to use.

Follow up Appointments:

Client will be encouraged to maintain consistent engagement in outpatient therapy, medication management, and recovery supports upon discharge. Continued participation in 12-step programming is recommended, including obtaining a sponsor to increase accountability and support recovery maintenance.

Treatment Plan Review Note

This treatment plan update is to reflect ■ moving to IOP 3 days per week effective 5/16.

Client continues to meet medical necessity for ongoing IOP level of care due to persistent high risk for relapse, ongoing emotional dysregulation, and significant environmental and behavioral risk factors. Since the last review, the client has demonstrated consistent attendance and engagement in treatment, including participation in both group and individual therapy sessions. She has made measurable progress in developing insight into her substance use disorder, identifying relapse triggers, and increasing awareness of maladaptive relational patterns; however, her insight remains inconsistently applied in real-world settings.

Client continues to present with complex co-occurring mental health conditions, including mood instability, anxiety, and impulsivity, which significantly impact her ability to regulate emotions and utilize coping strategies independently. She is actively processing trauma related to prior overdose events, which has contributed to increased emotional vulnerability, guilt, and distress. While the client is beginning to implement coping skills such as distress tolerance and emotional regulation, she continues to struggle with consistent application outside of structured treatment, particularly in response to interpersonal stressors and environmental triggers.

A critical clinical concern remains the client's recent decision to engage in employment within a bar environment, which represents a substantial increase in relapse risk due to constant exposure to alcohol, normalization of substance use behaviors, and direct access to high-risk situations. This decision reflects ongoing ambivalence in recovery and supports her current stage of change as contemplation. Given her history of chronic relapse, overdose, impulsivity, and substance use in similar environments, this factor significantly compromises her recovery stability and necessitates continued structured support.

The client's recovery environment remains only partially supportive. While she resides in a structured sober living setting that offers accountability and peer support, her difficulty maintaining routine, managing interpersonal dynamics, and adjusting to increased independence continues to limit the effectiveness of this support. Additionally, the client continues to experience moderate cravings and identifies multiple ongoing triggers, including emotional distress, boredom, and family-related stressors, further reinforcing the need for continued clinical intervention.

Despite these challenges, the client has demonstrated strengths including consistent treatment attendance, willingness to engage in therapeutic processing, and growing insight into the impact of her substance use. She has begun re-engaging in recovery supports and continues to benefit from structured programming, therapeutic interventions, and clinical monitoring. However, given the severity of her substance use history, relapse patterns, and current high-risk circumstances, she does not yet demonstrate the stability or skill generalization necessary to safely transition to a lower level of care.

Based on current presentation, continued IOP treatment remains medically necessary to provide ongoing relapse prevention, support skill development and application, address co-occurring mental health concerns, and assist the client in navigating high-risk environmental exposures. Treatment will continue to focus on strengthening coping strategies, increasing emotional regulation, addressing maladaptive relational patterns, enhancing motivation for recovery, and building a safe, structured, and sustainable recovery-oriented lifestyle.

Next Review Due

07/15/2026

ASAM Placement

ASAM Placement Summary Sheet

Date:

2026-05-16

Type (Check One):

☐ Admission

☒ Continued Stay

☐ Discharge

Dimension

	Level of Risk	Level of Care	Criteria Included/Comments
D1. Acute intoxication and/or withdrawal potential:	1	2.1	Client is not currently experiencing acute withdrawal symptoms and has maintained abstinence within a structured environment. However, her extensive history of polysubstance use, including alcohol, cocaine, benzodiazepines, and other substances, combined with multiple prior overdose events, places her at elevated risk in the event of relapse. Given her current exposure to high-risk environments and ongoing moderate cravings, continued clinical monitoring is necessary to ensure early identification of destabilization and to prevent progression to acute intoxication or withdrawal.
D2. Biomedical Conditions and Complications:	1	2.11	There are no current acute biomedical concerns; however, the client's history of multiple overdose events represents a chronic medical vulnerability. Additionally, she is actively processing past overdose experiences, which contributes to emotional activation and potential somatic stress. While stable at present, the risk of medical complications remains elevated in the context of relapse, necessitating ongoing monitoring within a structured level of care.

	Level of Risk	Level of Care	Criteria Included/Comments
D3. Emotional/Behavioral or cognitive conditions and complications:	2	2.1	Client presents with significant co-occurring psychiatric conditions, including Bipolar Disorder, Generalized Anxiety Disorder, Major Depressive Disorder, and ADHD. These conditions contribute to persistent emotional dysregulation, impulsivity, impaired distress tolerance, and difficulty maintaining behavioral consistency. She is actively processing trauma, including prior overdose experiences, which increases emotional vulnerability and contributes to guilt, shame, and reactivity. Client continues to struggle with regulating emotional responses, particularly in relation to family dynamics and interpersonal stressors. Her recent decision to engage in employment within a bar environment, despite awareness of relapse risk, further reflects impaired judgment and limited application of insight. Severity in this dimension strongly supports the need for continued structured therapeutic intervention.
D4. Readiness to change:	2	2.1	Client demonstrates engagement in treatment and verbalizes motivation for recovery; however, she remains in the contemplation stage of change. Although she is able to identify relapse risks and articulate recovery goals, her behavior reflects ongoing ambivalence, as evidenced by continued participation in high-risk activities, including employment in a bar setting. External structure, accountability, and therapeutic intervention remain primary motivators for engagement, indicating that she has not yet internalized recovery-oriented behaviors. Continued IOP care is required to support progression toward sustained commitment to change.
D5. Relapse, continued use or continued problem potential:	3	2.1	Client remains at high to severe risk of relapse, supported by a history of chronic relapse following periods of stability, prior overdose events, impulsivity, and difficulty managing emotional distress. She identifies multiple ongoing triggers, including boredom, lack of structure, interpersonal conflict, and family-related stressors. A critical escalation in relapse risk is her current employment in a bar environment, which provides constant exposure to alcohol, normalization of substance use, and direct access to substances. Combined with her inconsistent application of coping skills outside of structured settings and ongoing moderate cravings, the client requires continued intensive support to prevent return to use.
D6. Recovery Environment:	2	2.1	Client currently resides in a structured sober living environment, which provides accountability and peer support and serves as a protective factor. However, the effectiveness of this environment is limited by her difficulty maintaining routine, navigating interpersonal dynamics, and managing increased independence associated with employment. The addition of a high-risk occupational setting further compromises her recovery environment by introducing consistent exposure to substances. While supports are present, the overall environment remains unstable enough to warrant continued clinical oversight at the IOP level.

Indicate level of care recommended:

2.1

Indicate level of care received:

2.1

If recommended level of care is different from received, why?

n/a

Indicate the program or Facility referred to:

R3 Recovery Services

Brief Assessment of Recovery Capital (BARC-10)

Brief Assessment of Recovery Capital (BARC-10)

Instructions: On a scale of 1 (Strongly disagree) to 6 (Strongly agree), please indicate your level of agreement with the following statements.

1. There are important things to me in life than using substances.

☐ 1 Strongly Disagree
 ☐ 2 Disagree
 ☐ 3 Somewhat Disagree
 ☐ 4 Somewhat Agree
 ☒ 5 Agree
 ☐ 6 Strongly Agree

2. In general I am happy with my life.

☐ 1 Strongly Disagree
 ☐ 2 Disagree
 ☐ 3 Somewhat Disagree
 ☒ 4 Somewhat Agree
 ☐ 5 Agree
 ☐ 6 Strongly Agree

3. I have enough energy to complete the tasks I set myself.

☐ 1 Strongly Disagree
 ☐ 2 Disagree
 ☐ 3 Somewhat Disagree
 ☒ 4 Somewhat Agree
 ☐ 5 Agree
 ☐ 6 Strongly Agree

4. I am proud of the community I live in and feel part of it.

☐ 1 Strongly Disagree
 ☐ 2 Disagree
 ☐ 3 Somewhat Disagree
 ☐ 4 Somewhat Agree
 ☒ 5 Agree
 ☐ 6 Strongly Agree

5. I get lots of support from friends.

☐ 1 Strongly Disagree
 ☐ 2 Disagree
 ☐ 3 Somewhat Disagree
 ☒ 4 Somewhat Agree
 ☐ 5 Agree
 ☐ 6 Strongly Agree

6. I regard my life as challenging and fulfilling without the need for using drugs or alcohol.

☐ 1 Strongly Disagree
 ☒ 2 Disagree
 ☐ 3 Somewhat Disagree
 ☐ 4 Somewhat Agree
 ☐ 5 Agree
 ☐ 6 Strongly Agree

7. My living space has helped to drive my recovery journey.

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8. I take full responsibility for my actions.

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9. I am happy dealing with a range of professional people.

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 ☐ 2 Disagree
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 ☐ 2 Disagree
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42

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GAD-7

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1. Feeling nervous, anxious or on edge

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 ☒ Several days
 ☐ More than half the days
 ☐ Nearly every day

2. Not being able to stop or control worrying

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 ☐ Several days
 ☐ More than half the days
 ☐ Nearly every day

3. Worrying too much about different things

☐ Not at all
 ☒ Several days
 ☐ More than half the days
 ☐ Nearly every day

4. Trouble relaxing

- ☒ Not at all ☐ Several days ☐ More than half the days ☐ Nearly every day

5. Being so restless that it is hard to sit still

- ☒ Not at all ☐ Several days ☐ More than half the days ☐ Nearly every day

6. Becoming easily annoyed or irritable

- ☐ Not at all ☒ Several days ☐ More than half the days ☐ Nearly every day

7. Feeling afraid as if something awful might happen

- ☐ Not at all ☒ Several days ☐ More than half the days ☐ Nearly every day

GAD7 Score:

4

If you checked off any problems, how difficult have these problems made it for you to do your work, take care of things at home, or get along with other people?

- ☐ Not difficult at all ☒ Somewhat difficult ☐ Very difficult ☐ Extremely difficult

Patient Health Questionnaire (PHQ-9)

PATIENT HEALTH QUESTIONNAIRE (PHQ-9)

Over the last 2 weeks, how often have you been bothered by any of the following problems?

1. Little interest or pleasure in doing things.

- ☐ Not at all ☒ Several Days ☐ More than half the days ☐ Nearly every day

2. Feeling down, depressed, or hopeless.

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3. Trouble falling asleep, staying asleep, or sleeping too much.

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4. Feeling tired or having little energy.

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8. Moving or speaking so slowly that other people could have noticed? Or the opposite - being so fidgety or restless that you have been moving around a lot more than usual

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9. Thoughts you would be better off dead or of hurting yourself in some way

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PHQ-9 Score:

7

10. If you checked off any problems, how difficult have these problems made it for you to do your work, take care of things at home, or get along with other people?

☐ Not difficult at all ☒ Somewhat difficult ☐ Very difficult ☐ Extremely difficult

Columbia Suicide Risk Assessment

Columbia Suicide Risk Assessment

Ask questions 1 and 2. If both are negative, proceed to 6 "Suicidal Behavior" section. If the answer to question 2 is "yes", ask questions 3,4, and 5.

1. Wish to be Dead

- Client endorses thoughts about a wish to be dead or not alive anymore or wish to fall asleep and not wake up.

Have you wish you were dead or wished you could go to sleep and not wake up since last contact?

☐ Yes ☒ No

2. Non-Specific Active Suicidal Thoughts

- General, non-specific thoughts of wanting to end one's life/commit suicide without thoughts of ways to kill oneself/associated methods, intent, or plan during the assessment period.

Have you actually had any thoughts of killing yourself since last contact?

☐ Yes ☒ No

3. Active Suicidal Ideation with Any Methods (Not Plan) without Intent to Act

- Client endorses thoughts of suicide and has thought of at least one method during the assessment period. This is different than a specific plan with time, place, or method details worked out

Have you been thinking about how you might do this?

☐ Yes ☒ No

4. Active Suicidal Ideation with Some Intent to Act, without Specific Plan

- Active suicidal thoughts of killing oneself and subject reports having some intent to act on such thoughts.

Have you had these thoughts and had some intention of acting on them?

☐ Yes ☒ No

5. Active Suicidal Ideation with Specific Plan and Intent

- Thoughts of killing oneself with details of plan fully or partially worked out and subject has some intent to carry it out.

Have you started to work out or worked out the details of how to kill yourself? Do you intend to carry out this plan?

☐ Yes ☒ No

6. Suicidal Behavior

- A potentially self-injurious act committed with at least some wish to die, as a result of the act. Intent does not have to be 100%. If there is any intent/desire to die associated with the act, then it can be considered an actual suicide attempt. There does not have to be any injury or harm, just the potential for injury or harm. Also, if someone denies intent to die, but they thought that what they did could be lethal, intent may be inferred.

Have you done anything started to do anything, or prepared to do anything to end your life since last contact?

☐ Yes ☒ No

Summary and Recommendations

1. Overall Risk Assessment:

none

2. Additional Recommendations:

none

Treatment Plan LOC addendum

Level of Care

☐ PHP ☐ IOP 5 ☒ IOP 3 ☐ OP ☐ Other:

Group sessions

☐ 5 Groups per week ☐ 4 Groups per week ☒ 3 Groups per week ☐ 2 Groups per week ☐ 1 Group per week ☐ Other:

Individual sessions

☒ 1 session per week

☐ 2 sessions per week

☐ Other:

[REDACTED]

[REDACTED]

May 22 2026 12:45PM

Staff

[REDACTED]

Marleigh Johnson, MS, LPC
May 20 2026 8:16AM

Treatment Plan: 04/24/2026 - 04/24/2027

Problem: Substance Use Disorders

Diagnostic Impression:

04/24/2026 - [F14.20] Cocaine use disorder, Severe

04/24/2026 - [F10.20] Alcohol use disorder, Severe

04/24/2026 - [F19.20] Other (or unknown) substance use disorder, Severe

04/24/2026 - [F13.20] Sedative, hypnotic, or anxiolytic use disorder, Severe

Behavioral Definitions:

04/24/2026 - Client demonstrates a pattern of compulsive substance use (including alcohol, cocaine, benzodiazepines, and other substances) characterized by loss of control, increased tolerance, and continued use despite significant negative consequences, including legal issues, relationship strain, and medical complications.

04/24/2026 - Client exhibits high relapse risk behaviors, including impulsivity, poor decision-making, environmental vulnerability (peer influence, boredom), and inconsistent engagement in recovery supports, contributing to repeated relapse episodes following periods of sobriety.

04/24/2026 - Client demonstrates impaired functioning related to substance use, including emotional dysregulation, difficulty maintaining stability in relationships and daily responsibilities, and reliance on substances as a maladaptive coping mechanism to manage internal distress.

Client Goal: Client will increase overall functioning by addressing co-dependent patterns, processing significant past experiences (including overdose), and building a structured, value-driven lifestyle.

Objective: [REDACTED] will identify and modify at least 3 co-dependent behaviors and demonstrate use of healthy boundaries in relationships.

Intervention: Therapist will utilize DBT interpersonal effectiveness skills and psychoeducation on co-dependency to support boundary-setting, assertive communication, and reduction of maladaptive relational patterns.

06/23/2026: Status - (Open)

07/15/2026: Status - (Extended) Client has begun identifying co-dependent patterns, particularly within family and interpersonal relationships, and has demonstrated emerging awareness of maladaptive behaviors. She has engaged in skill-building related to boundary setting, though implementation remains inconsistent due to emotional reactivity and difficulty managing interpersonal stressors. Continued focus on DBT interpersonal effectiveness skills is clinically indicated to support sustained behavioral change.

Objective: [REDACTED] will develop and implement a structured routine and complete at least 2 steps toward educational or vocational goals (e.g., exploring school, job applications)

Intervention: Therapist will assist [REDACTED] in creating a structured weekly schedule and utilize solution-focused and ACT-based interventions to support goal setting, accountability, and follow-through on education/employment tasks.

06/23/2026: Status - (Open)

07/15/2026: Status - (Extended) Client has taken steps toward vocational engagement, including obtaining employment; however, current employment in a bar setting presents a significant relapse risk due to direct exposure to substances. Client continues to struggle with maintaining a consistent daily structure outside of treatment programming, with difficulties in follow-through and routine stabilization. Ongoing support is required to assist client in developing a recovery-oriented schedule that reduces risk and promotes stability.

Objective: [REDACTED] will engage in group and individual sessions by processing relationship dynamics, emotional responses, and past experiences (including overdose), and will demonstrate application of learned skills (e.g., boundaries, distress tolerance) in real-life situations.

Intervention: [REDACTED] will engage in group and individual sessions by processing relationship dynamics, emotional responses, and past experiences (including overdose), and will demonstrate application of learned skills (e.g., boundaries, distress tolerance) in real-life situations.

06/23/2026: Status - (Open)

07/15/2026: Status - (Extended) Client is actively engaging in both group and individual therapy, processing trauma related to past overdose and ongoing relational stressors. She demonstrates insight into emotional triggers and has begun applying coping strategies; however, skill application remains inconsistent in real-world situations, particularly when experiencing heightened emotional distress. Continued therapeutic intervention is needed to increase generalization and independent use of learned skills.

Client Goal: [REDACTED] will develop sustained recovery by increasing acceptance of substance use disorder (Step 1), improving emotional regulation, and utilizing effective coping strategies to reduce relapse risk.

Objective: [REDACTED] will verbalize acceptance of powerlessness over substances and identify at least 3 personal consequences demonstrating unmanageability within 30 days.

Intervention: Therapist will utilize Motivational Interviewing (MI) and CBT techniques to explore ambivalence, challenge denial, and process Step 1 concepts, linking [REDACTED]'s substance use history to patterns of loss of control and unmanageability.

06/23/2026: Status - (Open)

07/15/2026: Status - (Extended) Client has begun acknowledging the impact of her substance use and can identify consequences related to relapse, including overdose history and interpersonal strain. While she demonstrates increasing awareness, she continues to exhibit ambivalence and remains in the contemplation stage, as evidenced by ongoing engagement in high-risk environments. Continued motivational interviewing is required to support movement toward full acceptance and commitment to recovery.

Objective: [REDACTED] will identify and implement at least 3 coping skills to manage cravings, emotional distress, and relapse triggers (e.g., boredom, relationships, impulsivity).

Intervention: Therapist will provide CBT-based relapse prevention and DBT-informed coping skills training (e.g., distress tolerance, urge surfing), and rehearse application of skills to identified high-risk situations.

06/23/2026: Status - (Open)

07/15/2026: Status - (Extended) Client has identified several coping strategies, including emotional regulation and distress tolerance skills, and is able to verbalize their use in session. However, implementation remains inconsistent outside of structured settings, particularly in response to triggers such as boredom, emotional distress, and environmental exposure. Client continues to report moderate cravings, indicating a need for ongoing skill reinforcement and relapse prevention planning.

Objective: [REDACTED] will actively participate in both group and individual therapy by discussing relapse triggers, sharing personal experiences related to substance use, and applying feedback from clinicians and peers to support recovery-oriented behavior.

Intervention: Therapist will facilitate processing in both group and individual sessions, encouraging [REDACTED] to share experiences, receive peer feedback, and integrate insights into individualized relapse prevention planning and 12-step engagement.

06/23/2026: Status - (Open)

07/15/2026: Status - (Extended) Client is actively participating in treatment and demonstrates willingness to share personal experiences, particularly related to relapse history, trauma, and interpersonal challenges. She is receptive to feedback and demonstrates growing ability to integrate peer and clinician input into her recovery process. Despite this progress, ongoing emotional dysregulation and environmental risk factors continue to interfere with consistent behavioral change, indicating the need for continued structured care.

[REDACTED]

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■■■

April 24, 2026 08:35 AM

Staff [REDACTED]

[Handwritten signature: Marleigh Johnson, MS, LPC]

Marleigh Johnson, MS, LPC

April 24, 2026 08:35 AM

----- END OF TX PLAN REVIEW -----

Client Name : ■■■ ■■■ ■■■

DOB : [REDACTED]

MRN : [REDACTED]